Pemigatinib (Pemazyre®)

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What is pemigatinib (Pemazyre®)?

Pemigatinib (Pemazyre®) is a type of targeted therapy drug called a cancer growth inhibitor.

Pemigatinib is used to treat bile duct cancer where:

- the cancer is locally advanced or has spread to other parts of the body and
- the cancer has a change in a gene called the FGFR2 gene.

It is given when other cancer drug treatments such as chemotherapy are no longer working.

It is best to read this information with our general information about targeted therapy drugs and the type of cancer you have.

Your cancer team will talk to you about this treatment and its possible side effects before you agree (consent) to have treatment.

More information about this treatment

This information is correct at time of publishing. But sometimes the types of cancer this treatment is used for, or treatment side effects, may change between revision dates.

You can talk to your cancer team if you want more detailed information about this treatment. Or visit the electronic Medicines Compendium (eMC) website, which has patient information leaflets (PIL) for individual drugs.

How pemigatinib is given

During a course of treatment, you will meet someone from your cancer team, such as a:

- cancer doctor
- specialist nurse

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- specialist pharmacist.

This is who we mean when we mention doctor, nurse or pharmacist in this information.

During your course of treatment, you will have regular blood tests. This is to check that it is safe for you to have treatment.

You will meet with a doctor, nurse or pharmacist before you have treatment. They will talk to you about your blood results and ask how you have been feeling. If your blood results are okay, the pharmacy team will prepare your targeted therapy.

Your course of treatment

You have pemigatinib as tablets so you can take it at home.

You usually take pemigatinib once a day every day for 14 days. You then have a break and do not take it for 7days. This pattern is then repeated for as long as you take it.

You usually take pemigatinib for as long as it is working well for you and side effects can be managed. Your cancer team will discuss your treatment plan with you.

You may be given tablets of different strengths. Always take them exactly as you are told to by your cancer team. This is important to make sure they work as well as possible for you.

Your cancer team may also give you anti-sickness drugs and other medicines to take home. Take all your capsules or tablets exactly as they tell you to.

Taking pemigatinib tablets

You can take pemigatinib tablets with food or between meals.

Make sure you:

- swallow them whole with a glass of water
- do not chew, break or crush them
- take them at the same time every day.

If you forget to take the tablets, and it is less than 4 hours since you should have taken it, take the missed dose as soon as possible. If it is 4 hours or more since you should have taken the tablet, do not take the tablet to make up for the missed dose. Take the next dose at the usual time. If you are unsure, contact the hospital on the 24-hour number you have been given for advice.

You should not eat grapefruit or drink grapefruit juice when taking pemigatinib. This is because it may increase the risk of side effects.

Other things to remember about your tablets:

- Keep them in the original package and at room temperature, away from moisture, heat and direct sunlight.
- Keep them safe, where children cannot see or reach them.
- If you are sick (vomit) just after taking them, do not take an extra dose. Take your next dose at the usual time.
- Do not throw away unused tablets. Return them to your cancer team at the hospital.

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About side effects

We explain the most common side effects of this treatment here. We also include some that are less common.

You may get some of the side effects we mention, but you are unlikely to get all of them. And you may have some side effects, including rarer ones, that we have not listed here.

Other cancer treatments may cause different side effects. If you are also having other cancer treatment, you may have other side effects.

Always tell your doctor, nurse or pharmacist about any side effects you have. They can give you:

- drugs to help control some side effects
- advice about managing side effects.

It is important to take any drugs exactly as explained. This means they will be more likely to work for you.

Serious and life-threatening side effects

Some cancer treatments can cause serious side effects. Sometimes, these may be life-threatening. Your doctor, nurse or pharmacist can explain the risk of these side effects to you.

Contact the hospital

Your doctor, nurse or pharmacist will give you 24-hour contact numbers for the hospital. If you feel unwell or need advice, you can call at any time of the day or night. Save these numbers in your phone or keep them somewhere safe.

Very common side effects

These side effects happen to 10 or more people in every 100 people (10% or more) who have this treatment.

Changes to blood phosphate levels

This treatment can cause changes to the levels of phosphate in your blood. Phosphate is a mineral. Phosphates have different functions in the body, including keeping your bones and teeth healthy. Phosphates are in foods such as:

- meat
- eggs
- fish
- cheese.

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You will have regular blood tests to check phosphate levels. Phosphate levels may be raised or lower than normal. A raised phosphate level is more common than a low level.

Symptoms of a raised phosphate can include:

- numbness
- tingling around the mouth
- muscle cramps
- confusion

Contact the hospital straight away on the 24-hour number if you have any of these symptoms.

If your phosphate levels are raised, your cancer team may suggest you make some changes to your diet. They may also prescribe medicine to reduce your blood phosphate levels.

Changes to blood sodium levels

This treatment can cause low sodium levels in the blood. This can cause symptoms such as:

- headache
- feeling sick (nausea)
- poor balance
- confusion
- seizures

Contact the hospital straight away on the 24-hour number if you have any of these symptoms. You will have regular blood tests while taking this treatment to check your blood sodium levels.

Feeling sick

Your doctor, nurse or pharmacist will prescribe anti-sickness drugs to help prevent or control sickness. Take the drugs exactly as they tell you to, even if you do not feel sick. It is easier to prevent sickness than to treat it after it has started.

If you feel sick, take small sips of fluid often and eat small amounts regularly. It is important to drink enough fluids. If you continue to feel sick, or if you are sick (vomit) 1 to 2 times in 24 hours, contact the hospital on the 24-hour number as soon as possible. They will give you advice. They may change your anti-sickness treatment. Let them know if you still feel sick.

Sore mouth and throat

This treatment may cause a sore mouth and throat. You may also get mouth ulcers. This can make you more likely to get a mouth or throat infection. Use a soft toothbrush to clean your teeth or dentures in the morning, at night and after meals.

Contact the hospital straight away on the 24-hour number, if:

- a sore mouth or throat affects how much you can drink or eat
- your mouth, tongue, throat or lips have any blisters, ulcers or white patches.

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They can give you advice, and mouthwash or medicines to help with the pain or to treat any infection. Follow their advice and make sure you:

- drink plenty of fluids
- avoid alcohol and tobacco
- avoid food or drinks that irritate your mouth and throat.

Diarrhoea

This treatment may cause diarrhoea. Diarrhoea means passing more stools (poo) than is normal for you, or having watery or loose stools. You may also have stomach cramps. If you have a stoma, it may be more active than usual.

If you are passing loose stools 3 or more times a day and this is not normal for you, contact the hospital as soon as possible on the 24-hour number. Follow the advice they give you about:

- taking anti-diarrhoea medicines
- drinking enough fluids to keep you hydrated and to replace lost salts and minerals
- any changes to your diet that might help.

They might also ask you for a specimen of your stool to check for infection.

Constipation

This treatment can cause constipation. Constipation means that you are not able to pass stools (poo) as often as you normally do. It can become difficult or painful. Here are some tips that may help:

- Drink at least 2 litres (3½ pints) of fluids each day.
- Eat high-fibre foods, such as fruit, vegetables and wholemeal bread.
- Do regular gentle exercise, like going for short walks.

If you have constipation, contact the hospital on the 24-hour number for advice. They can give you drugs called laxatives to help.

If you have not been able to pass stools for over 2 days and are being sick, contact the 24-hour number straight away.

Changes to your taste

Some foods may taste different or have no taste. Try different foods to find out what tastes best to you. You may also get a bitter or metallic taste in your mouth. Your doctor, nurse or pharmacist can give you advice. It might help to try:

- sucking sugar-free sour or boiled sweets
- eating cold foods
- eating sharp-tasting fresh fruit.

Taste changes usually get better after treatment ends. We have more information about coping with changes to taste.

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Dry mouth

This treatment can cause a dry mouth. Tell your doctor if this happens. The following tips may help:

- Add moisture to food to make it easier to swallow. For example, moisten foods with gravies and sauces.
- Take frequent sips of water or a sugar-free drink. Keep a glass of water by your bed and carry a drink with you when you go out
- Suck ice cubes or lollies. You can make lollies by freezing fresh fruit juice or squash in ice cube trays or lolly containers with sticks.

Hand-foot (palmar-plantar) syndrome

This treatment can affect the palms of your hands and the soles of your feet. This is called palmar-plantar or hand-foot syndrome.

If you have white skin these areas may become red. If you have black or brown skin, these areas might get darker.

The skin on the palms of your hands and the soles of your feet may:

- be sore
- be painful, tingle, or swell
- peel, crack or blister.

If you have any of these symptoms, contact the hospital straight away on the 24-hour number. They can give you advice. This is especially important if you have any broken skin or if walking is difficult. They can prescribe creams and painkillers to help.

You can care for your hands and feet by:

- keeping your hands and feet cool by washing in cool water
- gently moisturising your hands and feet regularly
- wearing gloves to protect your hands and nails when working in the house or garden
- wearing loose cotton socks and avoiding tight-fitting shoes and gloves.

Nail changes

This treatment can affect your nails. They may break more easily or be a different colour. They may be painful or bleed. These changes usually disappear as the nails grow out after treatment. Sometimes nails can become loose or fall out.

If the skin around your nails becomes sore and swollen, contact the hospital straight away on the 24-hour number. These might be signs of an infection.

Tips to look after your nails:

- Keep your nails clipped short and clean.
- Avoid using very hot water when washing your hands or bathing.
- Moisturise your nails and cuticles regularly.
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- Avoid false nails, gels or other acrylics during treatment.
- Wear gloves to protect your nails when working in the house or garden.
- If your toenails are affected, wear well-fitting shoes, or shoes with open toes to cushion them.

Tell your doctor or nurse about any changes to your nails.

Hair loss

You might notice that your hair slowly gets thinner. But you are very unlikely to lose all the hair from your head. Your hair can also become more brittle. Your nurse can give you information about coping with hair loss. Your hair will usually go back to normal after treatment ends.

Dry skin

This treatment can cause your skin to be dry. Try using soap-free cleansers and unperfumed moisturising cream every day. Always tell your doctor, nurse or pharmacist about any skin changes. They can give you advice or prescribe creams or medicines to help if needed.

Dry eyes

This treatment can cause your eyes to be dry. Your doctor may prescribe eye drops for you to use while you are having pemigatinib.

Feeling tired

Feeling tired is a common side effect of this treatment. It is often worse towards the end of treatment and for some weeks after it ends. Try to pace yourself and plan your day so you have time to rest. Gentle exercise, like short walks, can help you feel less tired.

If you feel sleepy, do not drive or use machinery.

Joint pain

You may get pain in your joints. If this happens, tell your doctor, nurse or pharmacist. They can give you painkillers and advice. They can also tell you if any of the painkillers you usually take are suitable.

Tell them if the pain does not get better. Having warm baths and resting regularly may help.

Other side effects

These side effects happen to less than 10 in 100 people (less than 10%) who have this treatment. Some of them are much rarer than this but they are still important to know about. Rare means a side effect that happens to less than 1 in 1,000 people (less than 0.1%).

Eyesight changes

This treatment can affect your eyes and eyesight. You will have an eye test including a scan before you start treatment. These will be repeated every 2 to 3 months while you are taking pemigatinib.

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Contact the hospital straight away on the 24-hour number if you have any changes to your eyesight including:

- flashes of light or seeing black spots (floaters)
- blurred vision
- having a ‘shadow’ or ‘dark curtain’ across your vision

You may need to see an eye specialist urgently for treatment.

Skin nodules and ulcers

Rarely, pemigatinib can cause calcium deposits in the skin. This can cause painful nodules and ulcers in the skin. If you develop pain, nodules or ulcers anywhere on your body, contact the hospital straight away on the 24-hour number.

Changes to eyelashes

Pemigatinib can affect your eyelashes. Your eyelashes may grow longer than usual, or they might grow inwards.

Ingrown eyelashes can cause irritation, pain and redness of the eye. If you have any of these symptoms, contact the hospital straight away on the 24hour number. Your cancer team can arrange for an eye specialist to remove ingrown eyelashes.

Other important information

Blood clot risk

Cancer and some cancer treatments can increase the risk of a blood clot. Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- throbbing pain or swelling in a leg or arm
- reddening of the skin in the area – if you have black or brown skin, this can be harder to notice, but the skin might become darker
- suddenly feeling breathless or coughing.

Always call **999** if you have:

- chest pain
- difficulty breathing.

A blood clot is serious, but it can be treated with drugs called anticoagulants. These thin the blood. Your doctor, nurse or pharmacist can give you more information about preventing and treating blood clots.

Other medicines

Some medicines can affect how this treatment works or be harmful while you are having it. Always tell your cancer doctor, nurse or pharmacist about any drugs you are taking or planning to take, such as:

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- medicines you have been prescribed
- medicines you buy in a shop, pharmacy or online
- vitamins or supplements
- herbal drugs and complementary or homeopathic therapies
- recreational drugs – for example, cannabis.

Vaccinations

Cancer doctors usually recommend that people with cancer have vaccinations for flu and coronavirus (covid). They may also recommend other vaccines, such as Shingrix® for shingles. These all help reduce your risk of serious illness from these infections. Most people can have these vaccines, including people with weak immune systems.

You should not have live vaccines if your immune system is weak. This includes if you are having or recently had chemotherapy, radiotherapy or other cancer treatments that affect your immune system. Live vaccines can make you unwell because they contain a very weak version of the illness they protect you against. There are several live vaccines, including the yellow fever vaccine.

It is important to ask your doctor, nurse or pharmacist for advice about having vaccinations. They can explain what vaccines are right for you and when it is best to have them.

Contraception

Your doctor, nurse or pharmacist will advise you not to get pregnant or make someone pregnant while having this treatment and for some time afterwards. The drugs may harm a developing baby. It is important to use contraception to prevent pregnancy. Follow their advice about:

- what types of contraception to use
- how long after treatment you should continue to use contraception.

Breastfeeding

You are advised not to breastfeed while having this treatment, or for some time after treatment ends. This is because the drugs could be passed to the baby through breast milk.

Your doctor, nurse or pharmacist can give you more information.

Fertility

Some cancer drugs can affect whether you can get pregnant or make someone pregnant. If you are worried about this, it is important to talk with your doctor before you start treatment.

Medical and dental treatment

If you need medical treatment for any reason other than cancer, always tell the healthcare professional that you are having cancer treatment. Give them the contact details for your cancer doctor or cancer team so they can ask for advice.

If you have appointments with a dentist, always tell them you are having cancer treatment. Talk to your cancer team before you have any dental treatment.

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About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References

Visit the electronic Medicines Compendium (eMC) to download a Patient Information Leaflet (PIL) for more detailed information. The leaflet lists all known side effects.

Cancer Information Development team

Author

Date reviewed

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Our cancer information meets the PIF TICK quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about how we produce our information.

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

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We use gender-inclusive language and talk to our readers as ‘you’ so that everyone feels included. Where clinically necessary we use the terms ‘men’ and ‘women’ or ‘male’ and ‘female’. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information here.

How we can help

How to print a PDF from this website

Instructions on how you can make a PDF version of this page using the print function on your browser.

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.

Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.

Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don’t want to call.

Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.

Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help

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